Basic Diagnostic and Coding Procedures

A nine-step guide to diagnosis with the DSM-5-TR

Dr. Brian Campbell, Ph.D. · Belief Counseling · Counselors' Room

1 2 3 4 5 6 7 8 9

Why diagnostic codes matter

1

Medical records

Codes give every client record a clear, standard description of the diagnosis.

2

Research and statistics

Standard codes let clinics, researchers, and health agencies count and compare conditions.

3

Insurance payment

Medicare, Medicaid, and private insurers use the codes to decide what they will pay for.

The coding system behind the DSM-5-TR

ICD-10-CM

The code set listed in the DSM-5-TR and used for billing in the United States

What ICD stands for

International Classification of Diseases, Tenth Revision, Clinical Modification

What about ICD-11?

The World Health Organization adopted ICD-11 in 2022, but the DSM-5-TR and U.S. billing still use ICD-10-CM codes.

Working with insurance companies

Covered codes

Insurers pay only for certain diagnoses. Personality disorders, for example, are often not reimbursed.

Who files the claim

Clients can submit their own receipts with your code, or your office can file claims for them.

In network or out

Joining an insurer's panel makes you an in-network provider. Some insurers also pay out-of-network providers.

Medicare and Medicaid

Licensed professional counselors can now bill Medicare and Medicaid.

Preauthorization

Some services, such as psychological testing, need approval before you provide them.

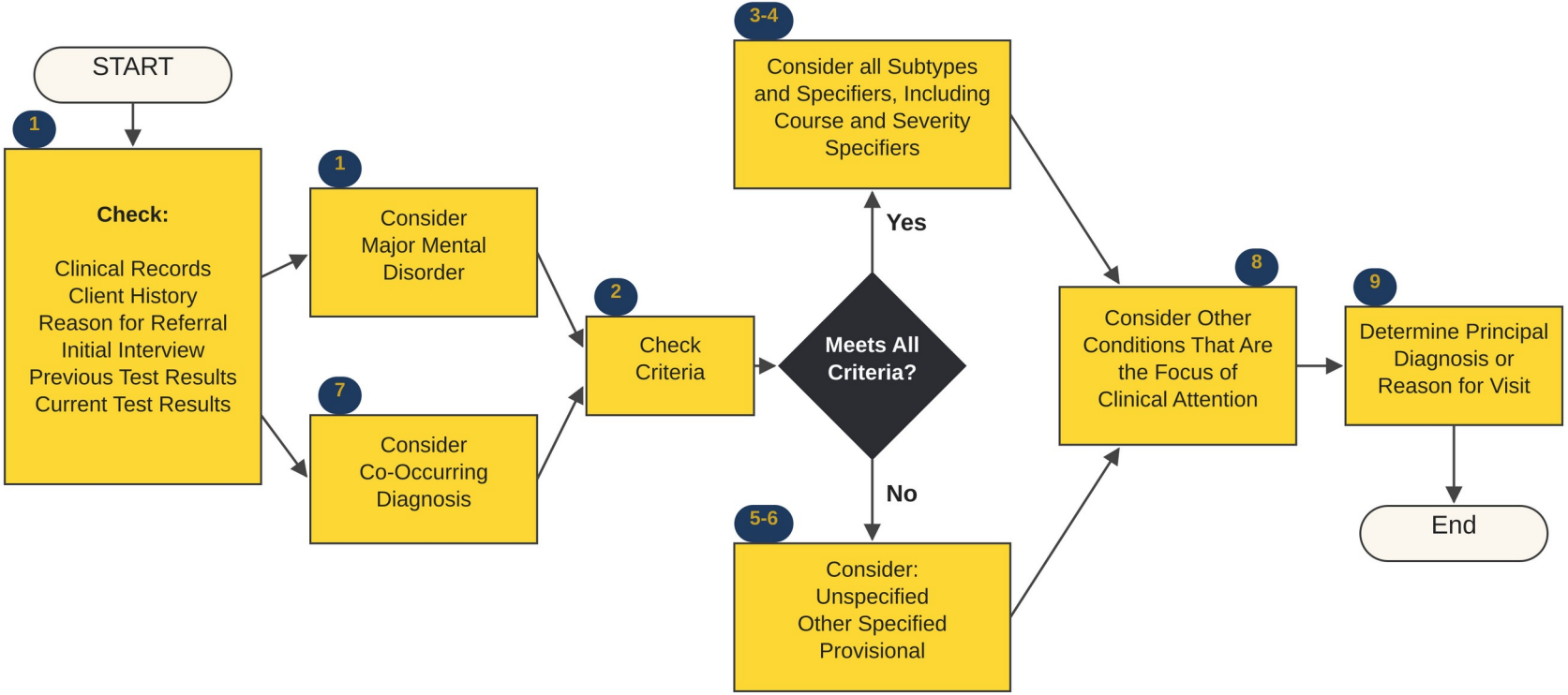
Electronic billing

Most insurers want claims filed electronically, using billing software or a billing service.

The nine-step diagnostic process

1. Explore for a major mental disorder
2. Check every criterion
3. Add subtypes and specifiers
4. Add course and severity
5. Consider other specified or unspecified
6. Consider a provisional diagnosis
7. Look for co-occurring disorders
8. Identify other conditions (Z codes)
9. Choose the principal diagnosis

The diagnostic flow chart



Step 1 · Explore for a major mental disorder

1

Gather everything you know about the client before deciding anything.

- **Background information**
- **Client history**
- **Reason for referral**
- **Initial interview**
- **Prior test data**
- **Current test data**

See it on the flow chart →

Step 2 · Check every criterion

THE RULE

A diagnosis is made only when every required criterion for the disorder is met.

Checking the criteria is the heart of the diagnostic process.

Go back to the record

Background data, test results, and clinical notes

Ask the client

Further questions about a specific criterion

Don't skip ahead

If a criterion is uncertain, find out before you diagnose

See it on the flow chart →

Step 3 · Add subtypes and specifiers

Subtypes

Marked in the criteria by the instruction “specify whether.”

Specifiers

Marked by “specify” or “specify if.” A disorder may have more than one.

EXAMPLE

F43.10 Posttraumatic stress disorder, with dissociative symptoms, with delayed expression

“with dissociative symptoms” is the subtype · “with delayed expression” is the specifier

See it on the flow chart →

Step 4 · Add course and severity

Course

- Persistent
- In partial remission · In full remission

Severity

Mild | Moderate | Severe | Extreme

EXAMPLE

F94.2 Disinhibited social engagement disorder, persistent, severe

Only with full criteria

Don't add course or severity to a provisional diagnosis.

See it on the flow chart →

Step 5 · Other specified or unspecified

Unspecified

The client looks like they have the disorder but doesn't meet every criterion, and you choose not to say why.

EXAMPLE

F44.9 Unspecified dissociative disorder

Other specified

You know why the criteria aren't met and choose to say so, by adding the reason after the name.

EXAMPLE

F32.89 Other specified depressive disorder, depressive episode with insufficient symptoms

See it on the flow chart →

Remembering the difference: the duck analogy

It looks like a duck in every way, but it doesn't have webbed feet, and webbed feet are one of the criteria for a duck.

Unspecified

You don't mention the feet. You simply call it: “Duck (unspecified)”

Other specified

You state the reason. You call it: “Other specified duck, duck without webbed feet”

See it on the flow chart →

Step 6 · Consider a provisional diagnosis

1. **Not enough information yet**
   It looks like the disorder, and more information would likely confirm it.

2. **The client can't give an adequate history**
   You believe the criteria would be met if the history were available.

3. **Duration isn't known**
   The diagnosis depends on how long a symptom has lasted, and you don't know yet.

EXAMPLE

F44.81

Dissociative identity disorder (provisional)

Write "(provisional)" in parentheses after the diagnosis.

See it on the flow chart →

Step 7 · Look for co-occurring disorders

First disorder

Confirmed through steps 1 to 6

+

Each additional disorder

Run it through the same steps, checking it against its own criteria.

Why it matters

Clients often have more than one disorder, and each one affects the treatment plan.

See it on the flow chart →

Step 8 · Identify other conditions (Z codes)

Use a Z code when the problem:

- Is a reason for the current visit
- Explains the need for a test, procedure, or treatment
- Helped start or worsen a mental disorder
- Should be considered in the overall treatment plan

- Relational
- Abuse and neglect
- Educational
- Occupational
- Housing
- Economic
- Social environment
- Legal system
- Access to health care
- Personal history

Tip: keep it to 1 to 4 Z codes.

See it on the flow chart →

Step 9 · Choose the principal diagnosis

Inpatient: principal diagnosis

The condition chiefly responsible for the client being admitted

Outpatient: reason for visit

The condition chiefly responsible for the services provided

List it first

Then list the rest in order of their focus in treatment.

Caused by a medical condition?

Coding rules list the medical condition first.

See it on the flow chart →

Writing the diagnosis clearly

My recommendations

- Code first, then the name of the disorder
- Severity right after the name
- Capitalize only the first word
- Specifiers in lowercase
- Mark the principal diagnosis or reason for visit

A COMPLETE OUTPATIENT DIAGNOSIS

F41.1 Generalized anxiety disorder (reason for visit)

F81.0 Specific learning disorder, mild, with impairment in reading

Z62.820 Parent-child relational problem (parent-biological child)

Z63.0 Relationship distress with spouse or intimate partner

Three examples

One disorder

F32.0 Major depressive disorder, single episode, mild, with anxious distress

With only one disorder, it's assumed to be the principal diagnosis.

Two disorders and a Z code

F41.1 Generalized anxiety disorder (principal diagnosis)

F81.0 Specific learning disorder, mild, with impairment in reading

Z64.0 Problems related to unwanted pregnancy

GAD has no specifiers.

No mental disorder

Z62.820 Parent-child relational problem (principal diagnosis)

Z64.0 Problems related to unwanted pregnancy

A Z code can be listed first.

Nine steps, one careful process

Gather → Check criteria → Specify → Address uncertainty → Sequence

1 2 3 4 5 6 7 8 9

Keep your DSM-5-TR by your side, and take it one step at a time.